

AI-Powered HMS Synthetic Clinical Dataset

Clinical Note

Patient Name	Trinh Quoc Hai	MRN	MRN-0034
DOB	1956-02-28	Gender	Male
Encounter Date	2024-08-01	Department	Neurology
Attending Provider	RN Jamie Owens	Status	active

Synthetic Data Warning: This document is generated for software testing, OCR parsing, chunking, embedding, access-control validation, and Graph RAG demonstrations. It is not valid medical advice and must not be used for patient care.

Subjective

Trinh Quoc Hai is a 70-year-old patient in Neurology presenting for follow-up of episodic neurologic symptoms with need for monitoring. PMH includes episodic headache or transient weakness, no persistent focal deficit on exam. Patient reports symptoms are stable overall. Denies severe chest pain, syncope, active bleeding, or new focal neurologic deficit unless otherwise documented. Current meds include levetiracetam 500mg BID; atorvastatin 20mg nightly; aspirin 81mg daily. Allergies: sulfonamide allergy - hives.

Objective

Vitals: BP 144/74 mmHg, HR 96 bpm, RR 16/min, Temp 37.0 C, SpO2 98% RA, weight 89 kg. Exam: alert, oriented, no acute distress. Cardiopulmonary exam without unstable findings. Abdomen soft, non-tender. Extremities show no acute deformity; edema varies by department-specific condition. Labs reviewed below; medication list reconciled.

Recent Lab Snapshot

Date	Analyte	Value	Unit	Reference Range	Status
2026-05-04	Creatinine	1.37	mg/dL	0.7-1.3	High
2026-05-04	eGFR	62.0	mL/min/1.73m2	>=60	Normal
2026-05-04	Glucose	126.0	mg/dL	70-99	High
2026-05-04	HbA1c	6.1	%	<5.7	High
2026-05-04	BNP	82.0	pg/mL	<100	Normal
2026-05-04	AST	38.0	U/L	10-40	Normal
2026-05-04	ALT	31.0	U/L	7-56	Normal
2026-05-04	Potassium	4.2	mmol/L	3.5-5.0	Normal
2026-05-04	Hemoglobin	13.8	g/dL	12.0-17.5	Normal

Assessment

Primary assessment: transient neurologic symptoms, seizure history, or migraine syndrome under evaluation. Patient is clinically stable for ongoing management. Risk considerations include renal dosing, medication safety, fall risk, and need for timely escalation if red-flag symptoms occur.

Plan

Document neuro checks and fall precautions. Review MRI/CT results when available. Escalate for new weakness, aphasia, seizure, or GCS decline. Provide patient education, update care team via SBAR, and document any access-relevant clinical justification before AI-assisted retrieval of PHI.

Detailed Care Coordination Notes

Problem List

1. transient neurologic symptoms, seizure history, or migraine syndrome under evaluation. 2. Medication safety monitoring. 3. Functional status and discharge readiness. 4. Follow-up coordination with Neurology.

Safety Triggers

Escalate to responsible clinician for SBP < 90, HR > 130, SpO2 < 92%, acute mental status change, active bleeding, severe pain, fever with neutropenia risk, or rapidly rising Cr/K.

AI Retrieval Tags

access_tags: read, summary, labs, medication, cardiology as applicable. Suggested document_type for ingestion: clinical_note.